

GULFSHORE VASCULAR & ENDOVASCULAR

412 Bayshore Boulevard, Suite 300 | Tampa, FL 33606 | (813) 555-0140 | Fax (813) 555-0141
NPI: 1497753102 | Tax ID: 59-5550142 | Fictional practice created for demonstration

PATIENT MEDICAL CHART

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1. Patient demographics

Patient name	David Okonkwo
Date of birth	November 2, 1954
Age at visit	70 years
Sex	Male
MRN	GSV-44802
Address	19 Magnolia Court, Brandon, FL 33511
Phone	(813) 555-0166
Emergency contact	Ada Okonkwo (spouse), (813) 555-0167
Primary care	Dr. Robert Feldman, MD, (813) 555-0114
Preferred language	English

2. Insurance

Primary insurance

Payer	Florida Blue, BlueMedicare PPO
Member ID	FBM880144219
Group	MA-FL-188
Prior authorization	Required for vein ablation
Auth telephone	1-800-555-0133

Secondary insurance. None on file.

3. Active problem list

ICD-10	Diagnosis	Onset	Status
I83.811	Varicose veins of right lower extremity with pain	2024	Active
I87.2	Chronic venous insufficiency (peripheral)	2024	Active
R60.0	Localized edema	2025	Active
I10	Essential hypertension	2011	Active
E11.9	Type 2 diabetes mellitus without complications	2015	Active

4. Current medications

Medication	Dose	Frequency	Indication
Losartan	50 mg	Daily	Hypertension

Medication	Dose	Frequency	Indication
Metformin	500 mg	Twice daily	Type 2 diabetes
Aspirin	81 mg	Daily	Cardiovascular prophylaxis

5. Allergies

Allergen	Reaction	Severity
Sulfa antibiotics	Rash	Mild

6. Visit notes — vascular surgery

Visit date: May 6, 2025 | **Attending:** James Whitfield, MD, RPVI | **Type:** New patient consult

Chief complaint. Right leg swelling and aching that limit work.

History of present illness. David Okonkwo is a 70-year-old warehouse lead. For a year he has had aching and end-of-day swelling of the right leg. The symptoms significantly interfere with activities of daily living; he cannot finish a warehouse shift on his feet, and he stopped coaching Saturday youth soccer because the leg swells. He has no venous ulcer, no bleeding from a varix, and no history of superficial thrombophlebitis. He has not worn medical compression. He does not smoke.

Examination.

Blood pressure 136/78 mmHg	BMI 28.1
Right leg Varicose veins along the medial thigh and calf. Pitting edema at the ankle. No pigment, no ulcer.	CEAP C3, symptomatic
Pulses Palpable at the foot, both sides	Left leg No edema and no varicosities

Assessment and plan.

Symptomatic right great saphenous insufficiency is suspected. CEAP C3. Ulcer, bleeding, and thrombophlebitis are absent.

Order venous duplex.

Fit 30-40 mmHg thigh-high graduated compression stockings today, May 6, 2025. Wear them every day while out of bed. Return after at least three months of daily use.

Electronically signed: James Whitfield, MD, RPVI — May 6, 2025 at 9:40 AM

Visit date: September 9, 2025 | **Attending:** James Whitfield, MD, RPVI | **Type:** Follow-up

Chief complaint. Four-month compression trial completed. Symptoms unchanged.

History of present illness. He was fitted with 30-40 mmHg thigh-high compression stockings on May 6, 2025. He wore them daily, including workdays and weekends. On September 9, 2025 the trial has lasted four months. Four months of compression therapy has not improved the aching or the swelling. He still cannot finish a warehouse shift, and he has not returned to coaching. There is still no ulcer, no bleeding, and no thrombophlebitis. He wants the vein closed.

Examination.

Compression 30-40 mmHg thigh-high, daily, May 6, 2025 through September 9, 2025	Right leg Persistent ankle edema. Varicosities unchanged. Skin intact.
Symptom change None. Aching and swelling are the same.	CEAP C3, symptomatic

Assessment and plan.

Right great saphenous reflux is confirmed on the June 2, 2025 duplex. Diameter is at least 3 millimeters. CEAP is C3.

The indication used is pain and swelling that significantly interfere with activities of daily living, after a failed compression trial of four months.

Order endovenous laser ablation of the right great saphenous vein.

7. Diagnostic reports

Venous duplex reflux study — June 2, 2025

Ordering physician	James Whitfield, MD, RPVI
Facility	Gulfshore Vascular Lab
Indication	Right leg edema and aching
Technique	Upright duplex

Interpretation. Right great saphenous vein: reflux lasting 1.1 seconds from the saphenofemoral junction through the mid-thigh. Great saphenous diameter 5.8 millimeters at the mid-thigh. Small saphenous vein is competent. No pathologic perforator. Deep veins patent, no thrombus. Left great saphenous vein is competent. Impression: right great saphenous reflux, diameter 5.8 millimeters.

Electronically signed: Elena Vasquez, MD, FACS — June 2, 2025

8. Planned procedure

Planned procedure	Endovenous laser ablation of the right great saphenous vein
CPT	36478 — laser ablation, first vein
Laterality	Right leg
Vein	Great saphenous vein. No accessory vein and no perforator.
Surgeon	James Whitfield, MD, RPVI
Facility	Gulfshore Vascular & Endovascular
Payer	Florida Blue BlueMedicare PPO
Authorization status	Not submitted. Request dated September 9, 2025.

Clinical indication summary

Right great saphenous reflux of 1.1 seconds and diameter 5.8 millimeters on June 2, 2025 duplex.

CEAP C3, symptomatic, with edema. No ulcer.

Pain and swelling significantly interfere with activities of daily living.

Daily 30-40 mmHg compression from May 6, 2025 through September 9, 2025 (four months) has not improved the symptoms.

No bleeding and no superficial thrombophlebitis.

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